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Public Comment Submission by CareCard

CMS-0042-NC

Re: CMS–0042–NC Request for Information; Health Technology Ecosystem

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Introduction

CareCard is a healthcare technology company purpose-built to close the affordability gap in prescription care. Our API and mobile-first solutions provide real-time cash pricing, financial assistance data, and pharmacy access tools across a national network—helping patients, providers, and health tech partners make better medication decisions at the point of care.

By embedding price transparency and affordability intelligence into the prescribing and pharmacy experience, CareCard reduces prescription abandonment, improves adherence, and strengthens health equity—particularly for low-income, Medicare, and underserved populations.

We are pleased to submit the following comments in support of CMS and ONC's shared goals to expand digital access, enable interoperability, and accelerate innovation across the health technology ecosystem. Below are responses to selected questions, organized by the CMS-provided labels.

Section B: Patients and Caregivers

PC–1 & PC–4: Health Management and Navigation Tools

Patients benefit significantly from tools that provide transparent and timely information about medication costs, discounts, and available financial assistance at the point of care.

Expanding digital capabilities to include seamless appointment scheduling, integration with delivery services, and medication adherence tracking can further enhance patient experience and outcomes.

CMS should encourage the adoption and development of digital health solutions that not only deliver clear pricing information but also actively reduce barriers to medication access, improving adherence and overall healthcare effectiveness for all Americans.

CareCard addresses one of the most fundamental—and often overlooked—health management barriers: the cost of medication at the point of dispense. Patients routinely leave prescriptions unfilled due to opaque pricing, benefit confusion, or unaffordable out-of-pocket costs.

Digital health tools must evolve beyond basic record aggregation or appointment reminders. Patients need:

- ✓ Real-time drug pricing across pharmacies
- ✓ Visibility into patient copay assistance programs (PAP) or discount programs
- ✓ Integrated delivery or pickup options
- ✓ Medication refill alerts and adherence support
- ✓ Visibility into their active health record(s) for authorized sharing with their providers (Physicians, Pharmacists)

We recommend CMS explicitly prioritize tools that *reduce out-of-pocket burden and improve medication access*, as these are foundational to overall care engagement and chronic disease management.

PC–5a: CMS Review of Digital Tools

CMS should consider establishing a streamlined, non-binding validation framework for digital health solutions, emphasizing usability, accessibility for seniors, and measurable improvements in patient outcomes such as medication pickup rates and refill adherence.

Encouraging broad participation across healthcare stakeholders in this type of validation initiative could help accelerate the adoption of impactful digital tools that enhance affordability, adherence, and overall patient health outcomes.

PC–6: Accessibility for Medicare Beneficiaries

Older adults frequently face challenges navigating fragmented patient portals, complex prescription benefit structures, and inconsistent drug pricing across pharmacies.

There is a critical need for mobile-friendly, enrollment-free solutions that simplify access to medication information and support better decision-making.

CMS should prioritize models that reduce friction for this population, focusing on intuitive design, price transparency, and ease of use to improve access and adherence for seniors - not just those integrated into enterprise portals or payer apps.

Section B.2: Data Access and Integration

PC–8 & PC–9: Access to CMS Data

While Blue Button 2.0 offers a valuable foundation, it lacks actionable pricing context and real-time formulary coverage insight.

We recommend extending CMS APIs to include:

- ✓ Formulary-tier drug coverage and patient-specific out-of-pocket estimates (using accumulators)
- ✓ Adherence flags (missed refills, delayed pickups)
- ✓ Integration with manufacturer copay programs and coupon eligibility

This would enable CareCard and others to develop tools that optimize for both clinical adherence and financial sustainability for patients.

Section B.3: Digital Identity

PC–14: Use of Digital Identity Credentials

We support the expansion of digital identity providers to streamline user authentication across CMS and third-party platforms. This would reduce password fatigue, increase API access adoption, and facilitate better data-driven patient support. However, CMS should promote interoperability among multiple credentialing solutions, not mandate a single federal identity provider. A competitive ecosystem (e.g., Login.gov, ID.me, CLEAR, etc.) fosters innovation and prevents a monopolized digital gatekeeper.

Any patient digital ID system should be explicitly limited in scope: used only to verify identity for accessing health records or services, not for broader surveillance, behavior profiling, or commercial reuse. CMS must articulae these limits clearly and enforce them via contract and policy.

Identity systems must embed zero-knowledge authentication, decentralized identifiers (DIDs), or similar privacy-preserving technologies where feasible. Patients should not have to trade privacy for access.

Section C: Providers

PR–4: Administrative Interoperability

CareCard could enhance provider-patient engagement by integrating with provider scheduling APIs and intake systems to help patients fill prescriptions and schedule necessary follow-ups—especially where medication affordability is a care gap.

Section D: Payers

PA–2 & PA–5: Enabling Patient-Facing APIs

Expanding payer APIs to include formulary coverage, patient-specific benefits, and real-time cost estimates will significantly enhance transparency and empower patients to make informed decisions about their care.

CMS can lead by example by requiring that patient access APIs be made available to third-party applications via standardized FHIR endpoints, ensuring consistent, secure, and equitable access to vital healthcare information across the ecosystem.

Conclusion

CareCard is committed to advancing medication adherence, price transparency, and equitable access to prescription care—especially for patients navigating high out-of-pocket costs or fragmented benefit structures.

To that end, we encourage CMS to take the following actions that would directly accelerate digital innovation and improve outcomes:

- ✓ **Expand the Blue Button 2.0 API** to include real-time formulary tiers, patient-specific out-of-pocket costs, and pharmacy pricing data - enabling developers to build tools that optimize adherence and affordability.
- ✓ **Establish a CMS-recognized validation program** for digital health solutions that focus on prescription cost navigation, non-benefit affordability pathways, and patient-centric adherence support.
- ✓ **Promote interoperable, unified digital identity standards.** CareCard supports broader adoption of secure, interoperable digital identity credentials to reduce friction for patients and caregivers accessing health information.

However, we urge CMS to take a principled, privacy-preserving approach that avoids centralization, protects user choice, and guarantees analog access for vulnerable populations.

- ✓ **Invest in real-world evidence infrastructure** to measure the impact of digital affordability tools on key metrics such as pickup rates, refill adherence, and total cost of care. We currently do this today.

CareCard stands ready to support CMS in building a more connected, transparent, and patient-accessible digital health ecosystem—one that meets patients where they are, regardless of benefit design or digital literacy. We appreciate the opportunity to comment and would welcome any opportunity to collaborate or participate in future pilot programs.

Sincerely,

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